



EXECUTIVE OFFICE OF THE PRESIDENT
OFFICE OF MANAGEMENT AND BUDGET
WASHINGTON, D.C. 20503

September 29, 2026
(Senate)

STATEMENT OF ADMINISTRATION POLICY

S.J. Res. 197 – Providing for congressional disapproval under chapter 8 of title 5, United States Code, of the rule submitted by the Centers for Medicare & Medicaid Services of the Department of Health and Human Services relating to "Patient Protection and Affordable Care Act, HHS Notice of Benefit and Payment Parameters for 2027; and Basic Health Program"

(Sen. Baldwin, D-WI, and 15 cosponsors)

The Unaffordable Care Act has been an unmitigated disaster for Americans since its passage. The Biden Administration further destabilized the individual market by allowing fraud and corruption to run rampant following drastic regulatory actions that eliminated common-sense eligibility verifications and other program integrity requirements. As a result, corrupted agents and brokers were able to exploit weak systems to enroll millions of people fraudulently, and often without their knowledge or consent, at massive expense to American taxpayers.

A recent study performed by the Department of Health and Human Services Office of the Assistant Secretary for Planning and Evaluation estimated that 5.6 million people were either improperly or outright fraudulently enrolled in plans they were not eligible for just last year. Efforts by the Trump Administration to crush fraud have reduced this number to 2.6 million in 2026, but challenges remain. For example, the Centers for Medicare and Medicaid Services recently uncovered one million enrollees without a Social Security number—a strong predictor of potential impropriety.

The 2027 Payment Notice, finalized in May, takes significant steps to restore program integrity to Federal and state-based marketplaces to help end Unaffordable Care Act fraud. It also takes important steps to provide financial relief to those subject to the law. The Payment Notice improves access to lower-cost qualified health plans, including catastrophic plans and plans that do not use networks to manage care access.

The rule also begins to correct one of the fundamental flaws of the Unaffordable Care Act by legalizing multi-year terms for catastrophic insurance plans. Currently, insurance contracts last no longer than a year and people often change their plans as they change jobs. The status quo leaves insurance companies without an incentive to invest in the long-term health of their enrollees and perpetuates a reactive “sick care” system instead of a proactive, prevention-first

health system. Under multi-year plan terms, issuers will have a strong financial incentive to invest in preventive care that is proven to keep populations healthy over this longer term, as they are financially liable for increased health care costs that could result from otherwise preventable illnesses.

The Trump Administration is deeply committed to reforms that expand access to health care, reduce fraud and corruption, and increase consumer control over their coverage options, and therefore strongly opposes passage of S.J. Res. 197. Passage of this resolution would lock in billions in annual taxpayer spending on outright fraud to the benefit of unscrupulous agents and brokers and large insurance conglomerates. It would roll back critical changes that will lower costs for the American people. And it will impede progress designed to fix incentives for health insurers to both Make America Healthy Again and keep America healthy for generations to come.

If S.J. Res. 197 were presented to the President, his advisors would recommend that he veto it.

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